

COVID-19 - Diagnosed or Suspected - Male (Child)

Algorithm 1143 | Child (0-17 years) | Gender at birth: Male

Definition

- COVID-19 symptom assessment decomposed from NHS.UK's published when-to-get-help guidance.
- Child patient.
- Male.

Initial assessment

1. [YES_NO] Has there been a positive COVID-19 test?
2. [DURATION] How long have symptoms lasted?
3. [OPEN_TEXT] Describe breathing, chest pain, alertness, fluid intake and urine, fever, oxygen saturation if already measured, and the main symptoms.
4. [OPEN_TEXT] Record age, pregnancy/postpartum, immune suppression, obesity, heart/lung/kidney disease, diabetes, vaccination, symptom-onset date, and safeguarding or safe-isolation limitations.
5. [CAREGIVER_OBSERVATION] What change has a safe parent or caregiver observed in alertness, breathing, eating or drinking, sleep, play, and normal activity?

Triage and redirect questions

1. Redirect → Anaphylaxis

Are sudden throat or tongue swelling, wheeze, faintness, or collapse the main concern rather than an infection developing over time?

Sudden systemic allergic-reaction features require the Anaphylaxis pathway.

1. Disposition level 100

Does the person seem very unwell or getting worse, have sudden chest pain, is so breathless they can't say short sentences at rest, are they coughing up blood, or have they collapsed, fainted, or had a first-time seizure, or developed a non-fading rash?

NHS.UK COVID-19 guidance lists these as emergency ambulance or Emergency Department criteria.

1. Disposition level 70

Without an emergency feature, is there worsening breathlessness, persistent fever or dehydration, oxygen saturation below the locally approved threshold, or is the patient an infant, pregnant/postpartum, older/frail, immunocompromised, or living with significant chronic disease?

Higher-risk patients and worsening respiratory or hydration symptoms need prompt assessment, including eligibility for time-sensitive treatment under current Qatar policy.

1. Disposition level 15

Are symptoms mild and improving in an otherwise low-risk older child or adult, with normal breathing, hydration and alertness, and none of the features above?

NHS.UK guidance recommends rest, fluids, and simple pain relief for mild COVID-19, with isolation guidance to protect others.

Advice

Emergency respiratory precautions

- Call Qatar 999 now, help the person sit in the easiest breathing position, use a mask only if tolerated and it does not impede breathing, and do not allow self-driving. Follow the call-handler's instructions. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: worsening breathing, blue/grey colour, collapse, confusion, or loss of consciousness.

Prompt Qatar COVID-19 assessment

- Use the current Qatar governance-approved in-person or telehealth pathway. Confirm current testing, isolation, antiviral eligibility, oxygen thresholds, pregnancy, pediatric and immunocompromise policy; this UAT protocol must not hard-code changing public-health rules.
- Call Back If: breathing worsens, oxygen falls, chest pain, confusion, poor intake, reduced urine, or collapse.

Home care for mild COVID-19

- Rest, drink plenty of water, take paracetamol or ibuprofen if uncomfortable, and try honey for cough (not for babies under 12 months). Stay away from others per local isolation guidance. Avoid lying flat while coughing.
- Call Back If: breathing difficulty develops; symptoms significantly worsen.

First aid

- Call Qatar 999 now, help the person sit in the easiest breathing position, use a mask only if tolerated and it does not impede breathing, and do not allow self-driving. Follow the call-handler's instructions. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- NHS.UK, "COVID-19 symptoms and what to do", <https://www.nhs.uk/conditions/covid-19/covid-19-symptoms-and-what-to-do/> (page last reviewed 21 March 2023)

Provenance

Shaped to mirror the licensed After-Hours Telehealth Triage Guidelines database structure. This is IST open-source guideline content derived from the cited public sources; it is not licensed STCC content. Question.DispositionLevel=null marks a See More Appropriate Guideline redirect and is not a disposition row. Demographic variants contain only source-supported differences; identical thresholds are explicitly documented rather than fabricated. Local destination mappings and TelemedicineEligible values require IST clinical governance approval. UAT DATA ONLY - NOT FOR REAL-PATIENT CARE OR PRODUCTION USE.

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